



SINUGEN TABLET

Description: Yellow, round and biconvex tablet. Diameter = 12.00mm.

Content

Each tablet contains:

Paracetamol	500.0mg
Pseudoephedrine hydrochloride	30.0mg
Chlorpheniramine maleate	2.0mg

This product contains PARACETAMOL.
Do not take any other paracetamol
containing medicines at the same time.

Indication

For the temporary relief of sinus congestion and pain. The antihistamine helps to relieve the irritation of allergic symptoms such as sneezing, itching and watery eyes.

Pharmacology

Paracetamol has analgesic and antipyretic actions weaker than aspirin. It has no anti-inflammatory properties. Pseudoephedrine hydrochloride, a stereoisomer of ephedrine, has sympathomimetic actions. It has less pressor activity and CNS effects. It is used as a bronchodilator and peripheral vasoconstrictor in preparations for the relief of nasal and bronchial congestion. Chlorpheniramine maleate, an alkylamine derivative with the properties and uses of antihistamines, one of the most potent antihistamines and generally causes less sedation than promethazine.

Small doses of paracetamol are rapidly and completely absorbed but the absorption of larger doses varies considerably. Paracetamol absorption appears to be mainly influenced by gastric emptying rate, the presence of food, especially carbohydrate. Pseudoephedrine hydrochloride is readily and completely absorbed from the gastrointestinal tract, with peak plasma concentration being achieved about 1 - 3 hours after oral administration. Chlorpheniramine maleate is readily absorbed after oral administration. Plasma half-life, after oral administration is 12 - 15 hours. Paracetamol is mainly conjugated to glucuronide and ethereal sulphate. Oxidation occurs to a small extent after the therapeutic doses and becomes more significant after large doses. Neonates produce more of the sulphate conjugate than the glucuronide whilst in the adult, the proportions of the two conjugates are either approximately equal or the glucuronide fraction is larger. N-demethylation of pseudoephedrine occurring to a small extent. Chlorpheniramine is extensively metabolized to form polar and non-polar metabolites. Two metabolites have been identified as a result of N-demethylation and those are monodesmethyl-chlorpheniramine and didesmethyl-chlorpheniramine. About 85% of a therapeutic dose is excreted in the urine in 24 hours. Of the excreted materials in the adult, 1 - 4% is unchanged, 20 - 30% is conjugated with sulphate, 40 - 60% is conjugated with glucuronic acid, 5 - 10% is composed of 3-hydroxy, 3-sulphate, 3-methoxyglucuronide and 3-hydroxy 3-sulphate metabolites, and 5 - 10% is composed of mercaptopuric acid and cysteine conjugates. Large amounts of the mercaptopuric acid and cysteine conjugates are excreted in overdosage. About 35% of a dose is excreted in the urine in 48 hours. The excretion of unchanged drug accounts for about 3 - 10% of the dose but it is increased by an increase in urinary pH and urinary flow. Both desmethylated metabolites are excreted in urine, less than 1% of a dose is excreted in the feces. About 70% of a dose is excreted in the urine in 36 hours with less than 1% as non-pseudoephedrine and the remainder as unchanged drug.

Contraindications

Do not use in patients with hypertensive diseases, incl. those with antihypertensive drugs, severe heart disease or hyperthyroidism or hypersensitivity to either any of the ingredients, or during, or for 14 days after, treatment with a monoamine oxidase (MAO) inhibitor.



Side effects/ Adverse effects

Paracetamol: Allergic reactions e.g. skin rash rarely occurs. It is generally safe. Pseudoephedrine: Urinary hesitancy or retention may occur in male patients with prostatic enlargement. Insomnia may occasionally occur. Drug eruptions with erythematous nummular patches has been very rare. Chlorpheniramine: Drowsiness may occur.

Cutaneous hypersensitivity reactions including skin rashes, angioedema, Stevens Johnson Syndrome/Toxic Epidermal Necrolysis have been reported.

Precautions

Cardiac disease, diabetes, glaucoma, impaired kidney or liver function and pregnancy, may impair ability to drive or operate machinery. Alcoholic drinks should be avoided. This product contains paracetamol. Do not take any other paracetamol containing medicines at the same time.

Warning:

-Not to be used in children less than 2 years of age.

-To be used with caution and on doctor's or pharmacist's advice in children 2 to 6 years of age.

-Allergy alert: Paracetamol may cause severe skin reactions. Symptoms may include skin reddening, blisters or rash. These could be signs of a serious condition. If these reactions occur, stop use and seek medical assistance right away.

Pregnancy and lactation

It should be avoided unless essential, especially during the first trimester.

Drug interactions

Symptomimetic agents e.g. decongestants, tricyclic antidepressants, appetite suppressants & amphetamine-like psychostimulants. MAO inhibitors-occasionally cause a rise in blood pressure. Bretylium, bethanidine, guanethidine, deprexine, methyldopa, α and β -adrenergic blocking agents - partially reverse the hypotensive action of the drugs.

Dosage and administration:

Adult and children over 12 years: 1-2 tablets, every 4 - 6 hours (not more than 8 tablets in one day)

Treatment of over dosage:

Symptoms: hepatotoxicity is caused by a single dose of 10-15gm paracetamol. Doses of 25gm or more is potentially fatal. Hypoglycemia, jaundice and metabolic acidosis may take 3 days to develop. Other symptoms may include abdominal pain, vomiting, sweating and hypotension. Treatment: Gastric lavage is initiated when patients are seen within 4 hrs of ingestion e.g. activated charcoal or 500ml oral iced mannitol 20%. If 15gm of paracetamol or more has been taken, antidotes has to be administered with either of the following:

A) 20% IV acetylcysteine stat while waiting for urine and plasma level tests results-initial 150mg/kg over 15 mins, followed by infusion 50mg/kg in 500ml 5% glucose over 4 hrs and 100mg/kg in IL 5% glucose over 16hrs.

B) 2.5gm oral methionine stat followed by 3 doses of 2.5gm q 4 hrs. For 3 yrs old, 1gm methionine q 4 hrs for 4 doses.

C) 140mg/kg oral acetylcysteine 5% as a loading dose, then 70mg/kg q 4hrs for 17 maintenance doses. Vomiting within 1 hr of administration requires repeat dose. This antidote is ineffective for more than 10 hrs overdosage.

Storage conditions

: Keep container tightly closed. Store in a cool, dry place below 30°C.
Protect from light. Keep away from children.

Presentation

: 1000's

Shelf-life

: 3 years

Manufactured & Distributed by

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